
The Global Burden of Mental Illness

Prevalence, Impact, and the Treatment Gap

SOURCE

Our World in Data / IHME Global Burden of Disease

YEAR

2019 Data

KEY INSIGHT Lifetime depression risk is substantial: 1 in 3 women and 1 in 5 men are affected globally.

LIFETIME RISK OF MAJOR DEPRESSION

1 in 3

Women

Will experience major depression
at some point in their lifetime

LIFETIME RISK OF MAJOR DEPRESSION

1 in 5

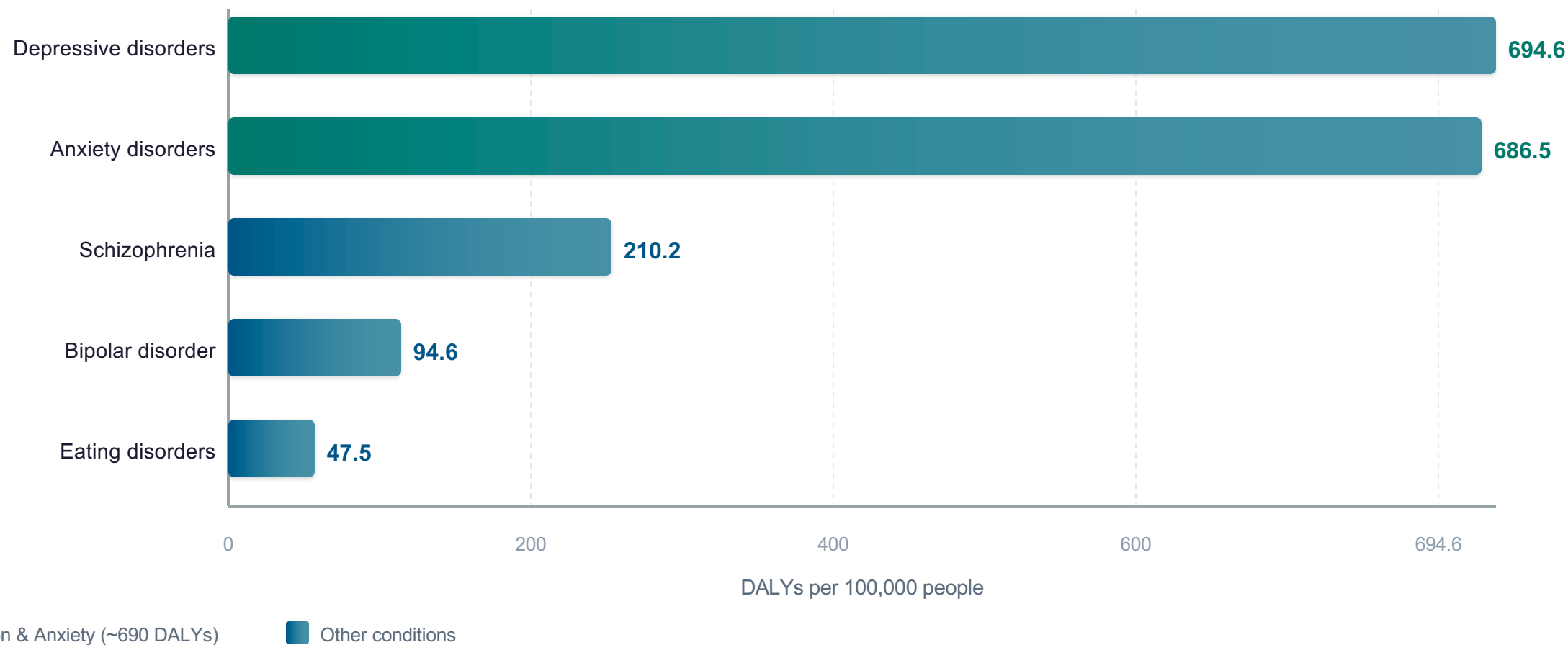
Men

Will experience major depression
at some point in their lifetime

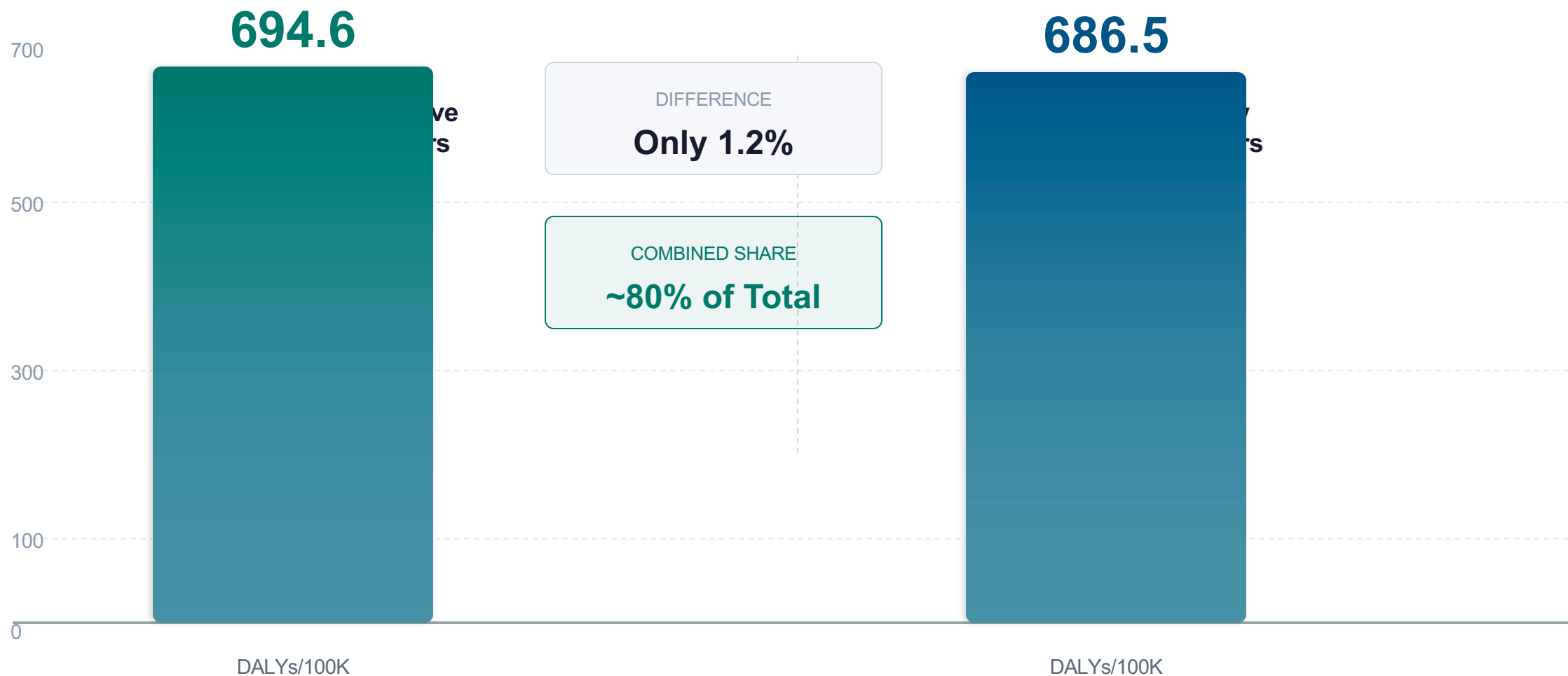
Other conditions such as schizophrenia and bipolar disorder are less common but carry severe individual impact.

Policy must address both common disorders and severe mental illnesses to reduce overall disease burden.

KEY INSIGHT Depressive and anxiety disorders together account for ~80% of mental health DALYs across these five categories.

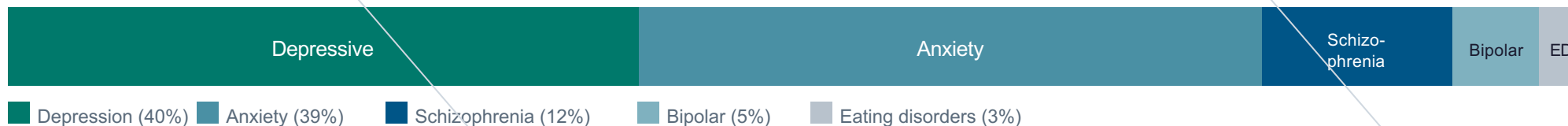


KEY INSIGHT Both disorders generate approximately 690 DALYs per 100,000 — nearly identical in magnitude.



KEY INSIGHT While population-level burden is lower, individual severity and disability from schizophrenia and bipolar are disproportionately high.

Proportional Share of Total Mental Health DALYs (1733.4 per 100K)



DALYs PER 100,000 PEOPLE

210.2

Schizophrenia

Reflects severe disability associated with
hallucinations, delusions, and cognitive impairment

DALYs PER 100,000 PEOPLE

94.6

Bipolar Disorder

Characterized by severe mood swings and
disruption to daily functioning and quality of life

Note: Schizophrenia and bipolar account for only ~17% of total mental health DALYs, yet carry high individual severity. Policy must maintain specialized care for these conditions while scaling interventions for common disorders.

KEY INSIGHT Mental illnesses are treatable, yet significant barriers prevent adequate care for millions worldwide.



Stigma

Discomfort disclosing symptoms

Many people feel uncomfortable sharing symptoms with healthcare professionals or people they know.



Access

Treatment often unavailable

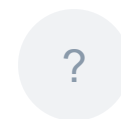
Treatment is often lacking, especially in low- and middle-income countries.



Quality

Even available treatment may fail

Even where treatment exists, it may be of poor quality, failing to meet clinical standards.



Underreporting

True prevalence is hidden

Discomfort with disclosure makes it difficult to estimate true prevalence — actual burden may be higher.

KEY INSIGHT

Coping strategies vary widely across countries and cultures — policy must be culturally sensitive.



Prescribed Medication

Common in higher-income settings

Includes antidepressants, anxiolytics, and other psychiatric medications under professional supervision and prescription.

Higher-income countries



Religious / Spiritual

Prevalent across many regions

Prayer, meditation, spiritual counseling, and other faith-based practices provide meaning, comfort, and community support.

Sub-Saharan Africa, Middle East, South Asia



Talking to Family & Friends

Widely reported globally

Informal social support through family conversations, peer support, and trusted social networks.

East Asia, Latin America, Europe

Diversity of coping strategies underscores that mental health responses are shaped by cultural, economic, and healthcare system factors. Policy interventions must account for these differences rather than applying a one-size-fits-all approach.

1

Depression and anxiety dominate

Together they represent ~80% of mental health DALYs across the five major categories.

2

Severe conditions carry disproportionate individual burden

Schizophrenia (210.2 DALYs) and bipolar disorder (94.6 DALYs) affect fewer people but cause severe disability.

3

Treatment exists but access and quality remain inadequate

The treatment gap is driven by stigma, poor access (especially in low-income countries), and inadequate quality of care.

4

Stigma suppresses help-seeking and accurate data

Discomfort disclosing symptoms means the true burden of mental illness is likely underestimated in global data.

5

Policy should prioritize scaling interventions for common disorders while maintaining specialized care

Evidence-based interventions for depression and anxiety should be scaled globally; schizophrenia and bipolar require sustained specialized services.